

COMPREHENSIVE RENAL FUNCTION & SERUM ELECTROLYTE PROFILE

Patient Name:	Karthik Ranganathan	Date of Exam:	September 14, 2026
MRN / Patient ID:	DEL-BMT-2026-9044	Age / Gender:	51 Yrs / Male
Blood Group & Rh:	B+ (Rh Positive)	Referring Doctor:	Dr. Gaurav Kharya, MD (Bone Marrow Transplant)
Clinical Diagnosis:	Post-Allogeneic BMT Day +28 (Calcineurin Nephrotoxicity Monitoring)	Specimen / Source:	Serum (Clot Activator SST)

Renal Biomarker	Patient Value	Reference Range	Unit	Evaluation
Serum Creatinine (Enzymatic)	1.04	0.70 - 1.30	mg/dL	Normal Glomerular Excretion
Blood Urea Nitrogen (BUN)	16.8	7.0 - 20.0	mg/dL	Normal Nitrogenous Clearance
Estimated GFR (CKD-EPI)	88.4	>= 60.0	mL/min/1.73m2	Normal Renal Clearance
Serum Sodium (Na+)	139.0	136.0 - 145.0	mEq/L	Euvolemic Electrolyte Balance
Serum Potassium (K+)	4.1	3.5 - 5.1	mEq/L	Normal Cardiac Stability
Serum Magnesium (Mg2+)	1.92	1.70 - 2.40	mg/dL	Normal Magnesium Level

CLINICAL INTERPRETATION & RECOMMENDATIONS:

- Serum creatinine (1.04 mg/dL) and eGFR (88.4 mL/min) demonstrate stable and preserved renal function. - No evidence of acute tubular necrosis, calcineurin inhibitor nephrotoxicity, or post-conditioning injury. - Electrolytes (Sodium 139 mEq/L, Potassium 4.1 mEq/L, Magnesium 1.92 mg/dL) strictly within normal therapeutic range. - Recommendation: Maintain adequate intravenous hydration; continue current Tacrolimus dosing.

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